

Q&A LOG
Program Executive Office (PEO) Defense Healthcare Management Systems (DHMS) Deployment Solutions
HT003826RE001
Date: 19 March 2026

Q#	DATE RECEIVED	Reference	Page#	Section Name	Paragraph/Line #	PROSPECTIVE VENDOR'S QUESTION	DATE OF REPLY	GOVERNMENT'S RESPONSE
1	5-Mar-26	PWS				Will subcontractors be required to hold a Facility Clearance for task orders issued under this IDIQ, or is it anticipated that clearance requirements will be determined on a task order basis? We did not see an FCL requirement specified at the IDIQ level and wanted to confirm how this will be handled.	19-Mar-26	The current IDIQ PWS does not require an Facility Clearance Level (FCL); however, any requirement for an FCL will be delineated to the task order level.
2	5-Mar-26	PWS				We are currently CMMC Level 1 self-assessed. We did not see a specific CMMC level requirement identified in the solicitation. Do you anticipate that CMMC Level 2 certification may be required for certain task orders, particularly as the CMMC program continues implementation beginning November 10, 2026?	19-Mar-26	CMMC Level 2 is anticipated for task orders.
3	5-Mar-26	PWS				My company is new to multiple award contracts. Will the Government explain how the PWS provided for the draft RFP will work for future task orders?	19-Mar-26	Each multiple award contract (MAC) holder will have the same PWS as an attachment to their respective contract award. The purpose of the IDIQ PWS is to provided the scope of end-to-end tasking allowable for the MAC. Specificity will be provided at each task order and incorporated into a task order PWS. IDIQs (D contracts) are often used when the Government anticipates recurring requirements (such as deployments) but cannot predetermine the precise magnitude or scale of the services required during the full performance period of the contract.
4	10-Mar-26	52.212-1	2	Bidscale Submission Portal Account Creation	Line 35	Recommend allowing a primary and backup Bidscale account for each offeror to reduce submission risk (single-account constraints can create avoidable schedule risk). Clarify support process and acceptable contingency steps if Bidscale access issues occur within 24 hours of due date.	19-Mar-26	4a and 4b. The Government will consider the recommendations as final revisions are incorporated for the combined synopsis/solicitation.
5	10-Mar-26	52.212-1	4	Page Size/Format; Line Numbering	Line 15	Clarify whether line numbering is required for all volumes and whether page limits apply per-volume or overall. Recommend allowing 0.5" margins and modern readable fonts (or explicitly confirm Times New Roman 12 is required) to reduce formatting disputes.	19-Mar-26	5a. Line numbering applies to all Volumes. 5b. Pursuant to 52.212-1 Section 4.0, Table 2 Page limits apply to the Volume unless an exception applies at 52.212-1 Section 3.1 to include title pages, cover pages, tables of contents, tab indexing, glossaries, and lists of tables and figures. 5c. The Government will consider the recommendation as final revisions are incorporated for the combined synopsis/solicitation.
6	10-Mar-26	52.212-1	2	Evaluation / Use of AI Tools	L105-L109 (anchor L107)	If AI tools are used to summarize proposals for evaluators, recommend adding guardrails: AI outputs are advisory only; final determinations are human; proposal data is not used to train models; retention limits for AI-generated summaries; and a documented review step for accuracy.	19-Mar-26	Pursuant to 52.212-1, Section 2.2, paragraph 4 "AI tools will be used solely as an administrative aid. All evaluation judgments and source selection decisions will be made by Government personnel."
7	10-Mar-26	PWS	4	Deployment Readiness Assessment	Line 143	Add a standardized 'Definition of Ready' checklist per site (network, identity, devices, training completion, change approvals) with a hard-stop gate prior to cutover. This reduces variability across sites and improves auditability.	19-Mar-26	Due to the unique requirements at the task order level, a standardized checklist is not feasible across all products for the IDIQ. See question 3.

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8	10-Mar-26	PWS	2	Training	L41-L45 (anchor L43)	Specify training effectiveness metrics beyond attendance (role-based competency checks, scenario-based validation, and completion evidence). Recommend accessible training artifacts (508-compliant) and multilingual support where applicable.	19-Mar-26	Specific training metrics may vary by task order based on mission needs and product delivery. Offerors are encouraged to propose unique solutions during these competitions IAW the evaluation criteria. See question 3.
9	10-Mar-26	PWS	9	Adoption and Change Management	Line 1	Recommend requiring an adoption plan with measurable outcomes (adoption KPIs, workflow adherence, ticket trends, and user satisfaction) and a post-go-live reinforcement cadence (office hours, refreshers, targeted coaching).	19-Mar-26	Offerors shall provide adoption plans based on mission needs and product delivery for respective task orders. See question 3. Specifically, for this proposal, refer to CDRL A003 Deployment Approach and 52.212-1 & 52.212-2.
10	10-Mar-26	PWS	5	Post-Deployment Stabilization Support	Line 28	Recommend defining a hypercare period (e.g., 30/60/90 days) with clear success exit criteria, escalation paths, and reporting cadence (issues, resolutions, root causes).	19-Mar-26	Post-Deployment Stabilization report may vary by task order based on mission needs and product delivery. Offerors are encouraged to propose unique solutions during these competitions IAW the evaluation criteria. See question 3.
11	10-Mar-26	PWS	5	Post-Deployment Stabilization Report; Lessons Learned	Line 32	Add a requirement for audit-ready deployment artifacts per site (readiness evidence, training completion, change approvals, cutover logs, and lessons learned) exportable as PDF + structured data for leadership and compliance review.	19-Mar-26	The Government will consider the recommendation as final revisions are incorporated for the combined synopsis/solicitation.
12	10-Mar-26	PWS	18	Security / Privacy / CUI Handling	Line 23	Recommend explicitly requiring least-privilege access, secrets management, and separation of duties for deployment actions. Include handling requirements for PHI/PII artifacts and secure storage/retention expectations.	19-Mar-26	Refer to PWS Sections 22 and 24 for minimum requirements.
13	10-Mar-26	PWS	14	Accessibility	Line 36	Recommend an explicit 508 compliance gate for deployment artifacts and end-user materials (training, job aids, PDFs). Include required testing (keyboard-only, screen reader) and documented remediation workflow.	19-Mar-26	Requirements may vary by task order based on mission needs and product delivery. See question 3.
14	10-Mar-26	PWS	5	Configuration, Validation, and Implementation; Interfaces	Line 10	Recommend clarifying dependencies and interfaces with existing healthcare systems at each site (identity, network, devices, local apps) and requiring an integration risk register and mitigation plan.	19-Mar-26	Due to the unique requirements at the task order level, clarifying dependencies and interfaces is not feasible across all products for the IDIQ. See question 3. Specifically, for this proposal and risk management, refer to CDRL A003 Deployment Approach and 52.212-1 & 52.212-2.
15	10-Mar-26	PWS	4	Change Control	L122-L126 (anchor L124)	Recommend including a formal change control process for deployment scope changes, site-specific variances, and configuration drift, with approvals and traceable decision history.	19-Mar-26	IAW PWS Section 3.1, the Quality Control Plan (QCP) shall establish a formal Change Control process to manage, document, and validate any modifications to the system or its configuration post-deployment. Additional requirements, if any, will be included in the task order PWS.
16	10-Mar-26	CDRL A003	1	Exhibit A003 Deployment Approach CDRL	N/A	Recommend that the Deployment Approach CDRL include: site readiness checklist, training plan + metrics, cutover plan, rollback plan, hypercare plan, and a governance/audit artifact package template.	19-Mar-26	Specificity such as this may vary by task order. See question 3.
17	10-Mar-26	PWS	48	Monthly Progress Report - SLA Metrics	Lines 1624–1626	Recommend defining measurable operational metrics (time-to-ready, training completion rate, cutover success rate, post-go-live ticket volume trend, time-to-stabilization) and requiring monthly dashboards.	19-Mar-26	Meaningful metrics may vary by task order. See question 3.
18	10-Mar-26	PWS	48	Monthly Progress Report - Personnel/Training Status	Lines 1613–1623	Recommend requiring a versioned runbook/knowledge base with approval gates and change history, so Tier 1/Tier 2 support can execute consistently across sites and reduce reliance on a small number of SMEs.	19-Mar-26	Specificity such as this may vary by product and task order. See question 3.
19	10-Mar-26	52.212-1&-2	2	Evaluation / Use of AI Tools (Admin Aid)	Line 52	Recommend limiting AI to assistive use cases that improve operational efficiency (ticket summarization, knowledge retrieval, reporting compilation) with mandatory human approval and full logging; prohibit autonomous changes in production without approvals.	19-Mar-26	The Government is unable to discern how this question relates to the 52.212-1 and 52.212-2 because the line number provided (52) references business days and proposal retention.
20	10-Mar-26	Solicitation	6	Factor 2 - Small Business Participation	Lines 254–283	Recommend clarifying how small business participation will be evaluated at the task-order level (teaming, subcontracting plans, and measurable participation targets) to encourage meaningful small business involvement.	19-Mar-26	The Government is considering the practicality of using Small Business Participation as an evaluation factor for the IDIQ competition.

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21	10-Mar-26	Solicitation	7	OCI / Proprietary Handling; Support Contractor Access	Lines 301–325	Recommend clarifying the process for marking proprietary information in feedback and confirm whether OCI declarations are required for draft-feedback submissions or only for the formal solicitation response.	19-Mar-26	OCI declarations are not required for the draft RFP feedback submissions. Proprietary information should not be submitted as part of feedback, since all industry questions and comments will be published to SAM.Gov.
22	10-Mar-26	52.212-1&-2	5	Factor 1 – Challenge Scenario 1a	223	Would Government please confirm and update the draft document clarifying whether, as part of the OCONUS medical center deployment, any datasets are expected to be exchanged with host-nation public health entities (e.g., for disease surveillance), including required data elements, cadence, standards, and sharing approvals.	19-Mar-26	No, this is not part of the Factor 1 Challenge Scenario.
23	10-Mar-26	52.212-1&-2	5	Factor 1 – Challenge Scenario 1a	229	Would Government please confirm and update the draft document clarifying the solution will be centrally hosted (CONUS) or regionally/edge hosted OCONUS, and establishing the latency assumptions/constraints to guide architecture and performance requirements.	19-Mar-26	For the purpose of Factor 1 Challenge Scenario 1a, the solution will be hosted CONUS. For the purpose of Factor 1 Challenge Scenario 1b, if the product is web hosted, it is located CONUS. If the product is not web hosted, it will be located at an individual site.
24	10-Mar-26	52.212-1&-2	5	Factor 1 – Challenge Scenario 1a	229	Would Government please confirm and update the draft document confirming any language and localization requirements (e.g., labels, patient instructions) and identifying applicable host-nation regulatory constraints that could impact design, content, and operations.	19-Mar-26	24a. For the purpose of Factor 1 Challenge Scenario, multilingual materials do not apply. 24b. For the purpose of Factor 1 Challenge Scenario, there are no regulatory constraints that could impact design, content, and operations.
25	10-Mar-26	52.212-1&-2	5	Factor 1 – Challenge Scenario 1b	244	Would Government please confirm and update the draft document indicating any constraints on remote administrative access into those networks, including requirements for jump hosts, multi-factor authentication (MFA), virtual desktop infrastructure (VDI), and approved administrative tools/controls.	19-Mar-26	For the purpose of Factor 1 Challenge Scenario 1b, offerors should assume disconnected, intermittent, and low-bandwidth (DIL) environments and no access to networks remotely.
26	10-Mar-26	52.212-1&-2	6	Factor 1 – Challenge Scenario 1b	241	Would Government please confirm and update the draft document indicating Service-specific networks/enclaves (by Service and environment) and identifying the designated network authority/owner for each to validate connectivity, access, and implementation decisions.	19-Mar-26	For the purpose of Factor 1 Challenge Scenario 1b, offerors should assume multiple Services are applicable.
27	10-Mar-26	52.212-1&-2	6	Factor 1 – Challenge Scenario 1b	249	Would Government please confirm and update the draft document by defining the minimum offline (potentially DDIL) clinical dataset required to meet mission needs (e.g., medications, allergies, notes, vitals, labs, imaging metadata), including required recency, completeness, and access constraints.	19-Mar-26	These details are not required for Factor 1 Challenge Scenario 1b submission.
28	10-Mar-26	52.212-1&-2	6	Factor 1 – Challenge Scenario 1b	241	Would Government please confirm and update the draft document clarifying the cost, licensing, and responsibility model for global multi-site deployments across EUCOM and INDOPACOM: whether each Service will fund and manage its own hardware/software (HW/SW) costs and licenses, or whether the Government expects an integrated service offering that bundles infrastructure, software, licensing, deployment, and sustainment across all sites and Services.	19-Mar-26	The Government is responsible for cost and licensing. For the purposes of Factor 1 Challenge Scenario, assume this would be provided. Offerors shall document any assumptions made within their proposed approach.
29	10-Mar-26	52.212-1&-2	6	Factor 1 – Challenge Scenario 1b	232	Would Government please update the draft document for each OpMed JOMIS deployed system (e.g. BATDOK-J, OpMed CDP, TBLD-M, etc.), with the primary installation platform(s) (rugged laptop, tablet, phone/handheld, edge/local server), required/approved OS and versions (Android, Windows, iOS/iPadOS, Linux), and current deployment counts by theater (EUCOM/INDOPACOM).	19-Mar-26	No, the Government will not provide deployment counts by theater. Offerors should assume that all platforms listed will be used as appropriate to Roles 1 through 3.
30	10-Mar-26	52.212-1&-2	6	Factor 1 – Challenge Scenario 1b	240	Would Government please confirm and update the draft document to include any targeted expansion counts/timeframe for JOMIS systems.	19-Mar-26	Targeted expansion counts and timeframes for JOMIS systems is not part of the Challenge Scenario.

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31	10-Mar-26	52.212-1&-2	6	Factor 1 – Challenge Scenario 1b	232	Would Government please confirm and update the draft document by specifying Bluetooth/sensor gateway constraints, encryption expectations for pairing, and data-at-rest and in transit requirements.	19-Mar-26	Offerors should include this in their proposal based on their assumptions.
32	10-Mar-26	52.212-1&-2	6	Factor 1 – Challenge Scenario 1b	234	Would Government please confirm and update the draft document by specifying any pre-approved wearable or other sensors JOMIS systems integrate in the current operations.	19-Mar-26	JOMIS systems do not currently integrate with any wearable or other sensors.
33	10-Mar-26	52.212-1&-2	6	Organizational Conflict of Interest (OCI) Declaration and Disclosure	280	Would Government please confirm and update the draft document clarifying what “team members” includes for OCI (e.g., subcontractors, vendors/original equipment manufacturers (OEMs), affiliates) and whether “all tiers” extends to indirect suppliers, to ensure consistent coverage and compliance expectations.	19-Mar-26	52.212-1, Contract Documents, OCI Declaration and Disclosure provides offerors an opportunity to identify any known, potential, or perceived OCIs so the Government may perform an independent assessment. “Teams” refers to anyone directly employed by an offeror or anyone employed by a subcontractor.
34	10-Mar-26	52.212-1&-2	7	Organizational Conflict of Interest (OCI) Declaration and Disclosure	303	Would Government please confirm what OCIs will exist with upcoming MHS GENESIS procurements that fall under HCDS including the PMO contracts (Technical Integration, Human Centered Design, Product Management) and the Services Contracts (End User Experience, Help Desk)?	19-Mar-26	At this time there is no known OCI with upcoming contracts. Potential OCI exists with awarded contracts that supports mid-office, back-office, or similar service contracts supporting internal business services. See question 33.
35	10-Mar-26		2	Attachment 16: Organizational Conflict of Interest Declaration and Disclosure	3	"1) had access to non-public information regarding solicitation HT003826RE001 that would assist my Company in obtaining the contract resulting from the solicitation, and 2) has not provided services under any Government contract that put my company in a position to influence the competition under solicitation HT003826RE001. " Would Government please confirm and update the draft document describing how the Government will identify organizational conflicts of interest, and providing clearer detail on the contracts, contract vehicles, or vendor relationships that could create an unfair competitive advantage which would result in a company unable to attest to this statement above.	19-Mar-26	See questions 33 and 34.
36	10-Mar-26	52.212-1&-2	6	Factor 2: Small Business Participation	267	Would the Government please clarify and update draft documents indicating that small business goals for this procurement are stand-alone and not cumulative. This is consistent with how small business goals have historically been structured, such that the total small business goal is 12% and any other small business category goals are subsets of the 12% and count toward the 12% goal rather than in addition to it. Additionally, requesting Govt explicitly confirm in the draft document that Total SB 30% is not applicable for evaluation or reporting under this requirement.	19-Mar-26	The Government is considering the practicality of using Small Business Participation as an evaluation factor for the IDIQ competition.
37	10-Mar-26	52.212-1&-2	6	Factor 2: Small Business Participation	267	Would Government please confirm and update the draft document to indicate the small business goals are based on total subcontracted dollars.	19-Mar-26	See question 36
38	10-Mar-26	52.212-1&-2	6	Factor 2: Small Business Participation	269	Factor 2 (page 6, line 256) allows offerors to claim small business credit for second- and third-tier subcontractors; however, it is unclear how offerors should apply for and substantiate that credit because the current reporting system (formerly eSRS) does not support second- or third-tier reporting and the Federal Acquisition Regulation (FAR) does not define a process. Would the Government please clarify and update the draft RFP to specify the required reporting method, reporting cadence, and required data elements needed to support second- and third-tier small business credit, and also confirm how the Government will verify that second- and third-tier subcontractors qualify as small businesses for credit.	19-Mar-26	See question 36

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39	10-Mar-26	52.212-1&-2	6	Factor 2: Small Business Participation	281	Would the Government please clarify and update the draft document accordingly indicating what evidence is required to demonstrate prior achievement of small business subcontracting goals, including whether ISRs are required. Given the volume of contracts held by many large business Offerors, compiling ISRs across all prior contracts could result in an excessive number of pages for Government review. If ISRs are required, would the Government please consider limiting submissions to no more than three representative contracts and confirm this approach is acceptable for evaluation.	19-Mar-26	See question 36
40	10-Mar-26	52.212-1&-2	7	Factor 2: Small Business Participation	329	In accordance to FAR 52.219-Pursuant to FAR 52.219-9, subcontracting plans must include estimated dollar amounts to be subcontracted. Given that this procurement is an IDIQ contract, would the Government please clarify the basis Offerors should use to determine the total contract value for purposes of preparing the subcontracting plan such as the IDIQ ceiling, a Government-provided estimated ordering value, or another specified baseline.	19-Mar-26	See question 36
41	10-Mar-26	52.212-1&-2	N/A	General	N/A	Would Government please confirm how task orders will be structured under this contract? For instance, will there be separate task orders for all deployment efforts in particular regions and/or will there be task orders for specific functions included in the scope to allow for centralization, standardization, and efficiency of those efforts (e.g., training, adoption and change management)?	19-Mar-26	Task order requirements may be grouped by product, region, or other logical manner which supports end-to-end deployments. See question 3.
42	10-Mar-26	PWS	13	SOFA	Section 11	Would Government please update the PWS to indicate locations where Government expects contractors to travel as needed and/or relocate for the performance of the work.	19-Mar-26	Services are applicable to Continental United States (CONUS) and Outside Continental United States (OCONUS) locations, covering various roles of care and facility types as specified in individual task orders.
43	10-Mar-26	52.212-1&-2	2	2.2 Proposal Submission Instructions	64-65	What is the anticipated proposal timeline for this opportunity and other opportunities in the HCDS portfolio?	19-Mar-26	The Government anticipates posting the combined Deployment Solutions synopsis/solicitation in March 2026 for ten days. Contractors should continue monitor SAM.Gov for other opportunities.
44	10-Mar-26	52.212-1&-2	5	52.212-2: Evaluation, Basis for Contract Award	213	How does the Government intend to evaluate the scalability of deployment models?	19-Mar-26	The Government is revising 52.212-1 to define scalable, innovative, feasible, and coherent.
45	10-Mar-26	52.212-1&-2	5	52.212-2: Evaluation, Basis for Contract Award	213	The challenge scenarios request innovative and scalable deployment approaches. Please clarify how the Government intends to evaluate the relative quality, innovation, or maturity of approaches.	19-Mar-26	See question 44.
46	10-Mar-26	52.212-1&-2	6	Factor 1, Challenge Scenario 1b Scope	240-241	For Challenge Scenario 1b (52.212-1, Page 6, Lines 240-241): The phrase "over five sites/units" is ambiguous. We recommend the government provide a maximum number of sites to clarify the requirement.	19-Mar-26	This will be revised in the combined synopsis/solicitation.
47	10-Mar-26	52.212-1&-2	7	Organizational Conflict of Interest	301-306	In the draft HCDS acquisition strategy published on December 22, 2025, the Government indicated potential OCI across the various workstreams. We request the government provide specific examples or guidance on what constitutes OCI as it relates to the Deployment Solutions contractor, product vendors, or other parties.	19-Mar-26	See Questions 33-34. An OCI with future contracts should be addressed as requested by that specific RFP. As the Government continues to refine future contract requirements, OCI will be further defined. As a general rule: To prevent an unfair competitive advantage, contractors and subcontractors performing "mid-office" or "back-office" support functions are generally prohibited from working on prime mission products. This is because their support role, which often involves providing strategic input and recommendations to the Government, could create an Organizational Conflict of Interest.

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48	10-Mar-26	52.212-1&-2	8	52.212-2: Evaluation, Basis for Contract Award	374	Will task orders be competed among all contract holders using FAR 16.505 procedures?	19-Mar-26	Unless an exception applies the intent is to compete all task orders are competed. See Revolutionary FAR Overhaul (RFO) 16.507-2.
49	10-Mar-26	52.212-1&-2	8	52.212-2: Evaluation, Basis for Contract Award	362-365	Over what period will the minimum guarantee be ordered (e.g., within first 12 months)?	19-Mar-26	The minimum guarantee will be obligated within the ordering period of the IDIQ.
50	10-Mar-26	52.212-1&-2	8	52.212-2: Evaluation, Basis for Contract Award	n/a	Will the Government assess the offeror's prior operational experience or past performance when evaluating the realism and feasibility of challenge scenario responses?	19-Mar-26	No, pursuant to 52.212-2 Factor 1 Challenge Scenario to be rated Acceptable, the offeror's proposal must meet the minimum requirements for each applicable sub-factor.
51	10-Mar-26	52.212-1&-2	8	Volume IV Contract Documents, Item (6)	351-355	Will "Addendum 3 - Pre-Award Identification and Assertion of Restrictions on Commercial Technical Data and Commercial Computer Software" be provided in a bidders library? What format/template should offerors use for data rights assertions if Addendum 3 is not available?	19-Mar-26	Addendum 3 Pre-Award Identification and Assertion of Restrictions on Commercial Technical Data and Commercial Computer Software will be provided in the complete terms and conditions within combined synopsis/solicitation. Addendum 3 was not provided in the draft RFP.
52	10-Mar-26	52.212-1&-2	9	52.212-2: Evaluation, Basis for Contract Award	413	Please clarify the specific objective criteria that will be used to determine whether a response to the challenge scenarios is feasible and coherent.	19-Mar-26	See question 44.
53	10-Mar-26	52.212-1&-2	9	52.212-2: Evaluation, Basis for Contract Award	373-376	What evaluation methodology will the Government use for task order competitions among multiple awardees (per FAR 52.212-1, Page 9, Lines 373-376)? For example, Best Value Trade-Off?	19-Mar-26	For each task order competition the Government will publish specific evaluation criteria for the unique requirement.
54	10-Mar-26	52.212-1&-2	9	FACTOR 1: CHALLENGE SCENARIO	420-465	Factor 1 Sub-Factors 1 & 2 require offerors to provide their approach to meet each line item in the sub-factor. Lines 421 and 430 state that: " <i>The proposal must describe...approach that ...</i> " Factor 1 Sub-Factors 3-5 requires offerors to demonstrate their approach. Lines 443, 454, and 462 state that: " <i>The proposal must demonstrate...approach that ...</i> " Does the Government want offerors to provide corporate experience or past performance demonstrating where they have applied their approach only for sub-factors 3-5 (and not 1 & 2)?	19-Mar-26	No, offerors should not provide experience or past performance in response to the Sub-Factors. The Government will revise Sub-Factors 3-5 accordingly.
55	10-Mar-26	52.212-1&-2	9	FACTOR 1: CHALLENGE SCENARIO	420-465	Factor 1 Sub-Factors 1 & 2 require offerors to provide their approach to meet each line item in the sub-factor. Lines 421 and 430 state that: " <i>The proposal must describe...approach that ...</i> " Factor 1 Sub-Factors 3-5 requires offerors to demonstrate their approach. Lines 443, 454, and 462 state that: " <i>The proposal must demonstrate...approach that ...</i> " To correctly address the different terms (describe vs demonstrate) used in the sub-factors, should offerors provide an approach to address sub-factors 1 & 2 and provide corporate experience to demonstrate where they have implemented their approach for sub-factors 3-5?	19-Mar-26	See question 54.
56	10-Mar-26	52.212-1&-2	10	M: Evaluation Criteria, Sub-Factor 4	459	How will the Government evaluate deployment and training approaches when determining whether a 'low-to-no touch' deployment model meets the Acceptable threshold?	19-Mar-26	The approach is feasible, coherent and has a reasonable expectation of successful performance.
57	10-Mar-26	52.212-1&-2	10	M: Evaluation Criteria, Sub-Factor 4	459	Will the Government provide CLINS to support purchase of ODCs required for deployment?	19-Mar-26	Yes, the combined synopsis/solicitation contains a schedule listing all anticipated CLINS to include ODCs and travel. For the Factor 1 Challenge Scenario offerors should assume hardware is provided.

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58	10-Mar-26	52.212-1&-2	10	M: Evaluation Criteria, Sub-Factor 4	454-455	How will the Government evaluate the extent to which an approach "leverages existing curriculum" (52.212-1, Page 10, Line 455)?	19-Mar-26	Offerors should take into account curriculum exists, meaning offerors do not have to develop curriculum in its entirety in their proposed approach. However, based on the offerors assumptions, revisions to the existing training curriculum may be required.
59	10-Mar-26	52.212-1&-2	9, 10, 11	52.212-2: Evaluation, Basis for Contract Award	N/A	To support preparation of compliant and consistent responses, could the Government clarify the criteria evaluators will use to determine whether each evaluation factor meets the Acceptable threshold?	19-Mar-26	Pursuant to 52.212-2 Factor 1 Challenge Scenario to be rated Acceptable, the offeror's proposal must meet the minimum requirements for each applicable sub-factor. For Supply Chain Risk, to be rated Acceptable the offeror's proposal must meet the following minimum requirements provide a feasible and coherent approach to identify, assess, and mitigate supply chain risks associated with hardware, software, and firmware, and confirm compliance with DFARS 252.204-7017, DFARS 252.204-7018, FAR 52.204-8 Deviation, FAR 52.204-26, 52.240-90, and 52.240-91 requirements. See question 36 for Small Business Participation.
60	10-Mar-26	52.212-1&-2	a	52.212-2: Evaluation, Basis for Contract Award	362-365	What are the IDIQ contract ceiling (maximum dollar value) and minimum guarantee (per awardee) for this procurement?	19-Mar-26	See clause 52.216-19 Order Limitations in the combined synopsis/solicitation when released.
61	10-Mar-26	52.212-1&-2	n/a	52.212-2: Evaluation, Basis for Contract Award	n/a	We recommend that the Government provide an estimated annual volume of task orders anticipated under the Deployment Solutions vehicle.	19-Mar-26	Due to the everchanging mission needs, an estimate of annual volume is not meaningful. An estimated volume is not necessary in either 52.212-1 or 52.212-2.
62	10-Mar-26	52.212-1&-2	n/a	52.212-2: Evaluation, Basis for Contract Award	n/a	Please clarify whether the Government expects offerors to provide specific deployment timelines, resource models, or staffing concepts in response to the challenge scenarios.	19-Mar-26	Offerors shall propose estimated deployment timelines based on the scenario provided, the offerors assumptions and the proposed response.
63	10-Mar-26	52.212-1&-2	n/a	52.212-2: Evaluation, Basis for Contract Award	n/a	Will price be evaluated at the task order level for deployment services.	19-Mar-26	Yes, price will be evaluated at the task order level.
64	10-Mar-26	52.212-1&-2	n/a	52.212-2: Evaluation, Basis for Contract Award	n/a	Will the Government establish standardized labor categories or rate structures that will apply to future task order competitions.	19-Mar-26	The Government does not anticipate establishing standardized labor categories or rate structures for the IDIQ.
65	10-Mar-26	52.212-1&-2	n/a	52.212-2: Evaluation, Basis for Contract Award	n/a	Does the Government anticipate task orders to include both Federal EHR deployment services and Operational Medicine deployment services under the same competition?	19-Mar-26	See question 41.
66	10-Mar-26	52.212-1&-2	n/a	52.212-2: Evaluation, Basis for Contract Award	n/a	Will task orders be structured around geographic deployments, facility-level deployments, or functional service areas such as integration, training, and sustainment?	19-Mar-26	See question 41.
67	10-Mar-26	52.212-1&-2	n/a	52.212-2: Evaluation, Basis for Contract Award	n/a	Will the Government provide surge support CLINS at the task order level?	19-Mar-26	A surge support CLIN is available for future task orders.

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68	10-Mar-26	PWS	11	5 Transition Period	380-408	Is there a current incumbent contractor(s) providing DHMS deployment services under this or a related contract vehicle? If so: - What is/are the incumbent(s) contract number(s)? - Will the incumbent provide transition support (documentation, knowledge transfer) to awardees? - What historical deployment data, lessons learned, or SOPs will be made available during the 30-day transition-in period (PWS Page 11, Lines 381-387)?	19-Mar-26	68a. While not a direct follow-on, aspects of this acquisition have been performed under N0003915D0044 (Leidos, Inc.), HT003821D0001 (Cherokee Nation), HT003823C0009 (Ellumen, Inc.), HT003824C0004 (Accenture Federal Services, LLC), and HT003825C0003 (Advancia Aeronautics, LLC). 68b. Applicable historical deployment data, lessons learned, or SOPs (if any) will be provided by task order.
69	10-Mar-26	PWS	18	16.4 Cybersecurity Requirements	622-627	What CMMC level (Level 1, Level 2, or Level 3) is required for this contract? Per PWS Page 18, Lines 622-627, the requirement references DFARS 252.204-7021 but does not specify the maturity level.	19-Mar-26	See question 2.
70	10-Mar-26	PWS	7,8	3.5.4 Training	n/a	Is the deployment contractor responsible for providing and maintaining the training environment(s) in coordination with the product vendors?	19-Mar-26	Any requirement for the contractor to provide and/or maintain a training environment will be specified at the task order.
71	10-Mar-26	Solicitation	2	2.2 Proposal Submission Instructions for Bidscale	64-69	Please confirm the file size limitations in Bidscale.	19-Mar-26	1.5 GB per file
72	10-Mar-26	Solicitation	3	2.4 Amendments to Solicitation	140-142	How should amendments to the solicitation be acknowledged? We recommend the amendments be acknowledged in the cover letter of the proposal response.	19-Mar-26	Amendments to the solicitation, if any, shall be acknowledged by digital signature on the SF 30 Box 15 by the offeror's authorized person to sign binding documents.
73	10-Mar-26	Solicitation	5	Section 3: Support Contractor Proposal Access Consent Form	n/a	Please provide a copy of the Support Contractor Proposal Access Consent Form. This was not attached in SAM.	19-Mar-26	The combined synopsis/solicitation will be revised to instruct offerors to include permission in the Cover Letter.
74	11-Mar-26	PWS	4	3.2 Deployment Planning and Execution	Lines 120-135	Observation: The PWS references deployment activities across multiple locations and operational environments but does not specify the anticipated deployment scale, concurrency, or operational tempo expected during the contract period of performance. Recommendation: We recommend providing additional context regarding expected annual deployment volumes, anticipated number of concurrent deployment waves, approximate number of users supported per deployment site, and expected geographic distribution of deployments (CONUS vs OCONUS). Value: Providing these parameters will enable offerors to design scalable deployment frameworks, develop realistic staffing models, and plan logistics, training, and site preparation activities in a manner that minimizes disruption to healthcare delivery operations while ensuring continuity of care during system transitions.	19-Mar-26	Specificity such as this may vary by product and task order. See question 3.

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75	11-Mar-26	PWS	4	3.2 Deployment Planning and Execution	Lines 120–135	Observation: The PWS describes deployment planning activities but does not explicitly address strategies for maintaining continuity of clinical workflows during system transition events. Large healthcare system deployments frequently require coordination with clinical leadership to ensure that patient care delivery is not disrupted during system cutover and activation periods. Recommendation: We recommend clarifying whether deployment approaches should include clinical workflow transition planning, coordination with site clinical leadership, and strategies for maintaining patient care continuity during go-live events. Value: Incorporating clinical workflow continuity considerations into deployment planning will reduce operational risk, support clinician adoption, and ensure that patient care delivery remains uninterrupted during system transitions.	19-Mar-26	Please refer to CDRL A003 Deployment Approach.
76	11-Mar-26	PWS	4	3.2 Deployment Planning and Execution	Lines 120–135	Observation: The PWS describes deployment execution activities but does not address governance structures used to coordinate enterprise-scale deployments across multiple facilities and operational environments. Recommendation: We recommend clarifying whether offerors should describe a deployment governance framework that includes centralized program management, site coordination mechanisms, and escalation pathways for resolving operational issues encountered during deployments. Value: Establishing a structured governance model will help ensure consistent deployment execution across geographically distributed sites, improve coordination between Government stakeholders and deployment teams, and support effective risk management during large-scale healthcare system rollouts.	19-Mar-26	Specificity such as this may vary by product and task order; an overall approach to governance is required by CDRL A003 Deployment Approach. See question 3. Specifically, for this proposal, refer to CDRL A003 Deployment Approach and 52.212-1 & 52.212-2.
77	11-Mar-26	PWS	4	3.2.1 Deployment Readiness Assessment	Lines 140–150	Observation: The PWS requires deployment readiness assessments but does not define the readiness framework or evaluation methodology used to determine site readiness prior to system implementation. Recommendation: We recommend clarifying whether a standardized readiness model will be applied that includes technical readiness (network infrastructure, hardware availability), operational readiness (workflow alignment and clinical governance), user readiness (training completion and adoption readiness), and cybersecurity readiness. Additionally, defining readiness scoring thresholds, remediation procedures, and Government approval gates prior to go-live would improve clarity. Value: Establishing a standardized readiness assessment framework will support consistent evaluation across deployment sites and reduce implementation risk associated with deploying clinical systems into environments that are not fully prepared.	19-Mar-26	Specificity such as this may vary by product and task order. See question 3.

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78	11-Mar-26	PWS	5	Operational Medicine Deployment	Lines 220–238	Observation: The PWS describes deployment of Operational Medicine solutions across expeditionary environments such as theater hospitals, naval vessels, and forward resuscitative sites; however, operational constraints within these environments are not clearly defined. Recommendation: We recommend clarifying expected operational conditions including network availability, power constraints, environmental conditions, mobility requirements, and logistics support available for deployed hardware. Value: Providing greater visibility into the operational environment will enable offerors to design resilient deployment architectures capable of supporting reliable clinical documentation and patient care continuity across the full continuum of battlefield and expeditionary medical operations.	19-Mar-26	Offerors shall assume the minimum operational conditions are available. Disconnected, Intermittent, and Limited (DIL) network environments are the expectation for Operational Medicine deployments. Specificity such as this will vary by product and task order.
79	11-Mar-26	PWS	6	3.3 Hardware Delivery and Connectivity	Lines 245–252	Observation: The PWS requires systems to operate in Disconnected, Intermittent, and Limited (DIL) network environments but does not define technical thresholds that characterize those environments. Recommendation: We recommend clarifying expected bandwidth thresholds, frequency and duration of connectivity disruptions, synchronization tolerance levels, and offline data capture expectations. Value: Defining these parameters will allow offerors to design architectures that preserve data integrity, enable reliable synchronization, and support continuous clinical operations despite connectivity limitations.	19-Mar-26	See question 78.
80	11-Mar-26	PWS	6	3.4 Medical Device Connectivity and Validation	Lines 228–231	Observation: The PWS requires integration with medical devices but does not identify device categories, interface standards, or integration responsibilities. Recommendation: We recommend clarifying expected device categories (e.g., imaging systems, laboratory analyzers, patient monitoring equipment, infusion devices), applicable interoperability standards such as HL7, FHIR, and DICOM, and responsibilities for device integration validation and vendor coordination. Value: Providing this information will enable offerors to design integration architectures that ensure safe and compliant interoperability between clinical systems and medical devices.	19-Mar-26	Specificity such as this may vary by product and task order. See question 3.
81	11-Mar-26	PWS	7	3.5 User Enablement Services	Lines 260–270	Observation: The PWS addresses training and user enablement but does not explicitly reference post-deployment stabilization activities commonly implemented following healthcare system go-live events. Recommendation: We recommend clarifying whether the deployment approach should include post-deployment stabilization or “hypercare” support periods during which deployment teams provide elevated operational support to resolve user issues, optimize workflows, and address system configuration adjustments. Value: Including structured stabilization periods following deployment will improve system adoption, reduce operational disruption, and allow healthcare personnel to transition more effectively to the new system environment.	19-Mar-26	See PWS Section 3.2.3 Stabilization and Transition.

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82	11-Mar-26	PWS	7	3.5.4 Training	Lines 260–270	Observation: The PWS outlines training requirements but does not specify the scale or scope of training activities expected during deployments. Recommendation: We recommend clarifying estimated user populations per deployment site, distribution of clinical and administrative roles requiring training, training delivery modalities, and whether Government-provided training materials will be available. Value: Providing this information will allow offerors to develop scalable training programs that accelerate adoption while minimizing disruption to healthcare operations.	19-Mar-26	Specificity such as this may vary by product and task order. See question 3.
83	11-Mar-26	PWS	7	Global User Support	Lines 270–280	Observation: The PWS requires support for a globally distributed user community but does not define workforce coverage expectations across time zones and operational environments. Recommendation: We recommend clarifying expected operational support hours, requirements for on-site versus remote support models, and surge staffing expectations during concurrent deployments. Value: Clarifying these expectations will enable offerors to design workforce models capable of supporting continuous healthcare operations across geographically dispersed medical facilities.	19-Mar-26	Specificity such as this may vary by product and task order. See PWS Section 3.5.3 Consideration of Global User Community.
84	11-Mar-26	PWS	9	3.5.5 Adoption and Change Management	Lines 280–284	Observation: The PWS requires monitoring of user adoption and satisfaction but does not specify performance metrics used to measure adoption success. Recommendation: We recommend defining measurable adoption metrics such as training completion rates, workflow utilization metrics, user satisfaction benchmarks, and operational performance indicators. Value: Clearly defined metrics will enable contractors to implement data-driven change management strategies that improve adoption outcomes and ensure clinical workflows transition successfully to the new system.	19-Mar-26	Specific user adoption metrics may vary by task order based on mission needs and product delivery. Offerors are encouraged to propose unique solutions during these competitions IAW the evaluation criteria.
85	11-Mar-26	PWS	10	3.6 Data Management and Migration	Lines 285–289	Observation: The PWS requires migration of clinical and administrative data but does not specify expected data volumes, legacy system sources, or validation requirements. Recommendation: We recommend clarifying the anticipated scope of data migration, legacy data repositories involved, and Government expectations regarding data validation and reconciliation processes. Value: Providing this information will enable contractors to design migration strategies that preserve clinical data integrity and minimize operational disruption during system transitions.	19-Mar-26	Specificity such as this may vary by product and task order. See question 3.
86	11-Mar-26	PWS	11	Cybersecurity / RMF Compliance	Lines 286–289	Observation: The PWS references compliance with the DoD Risk Management Framework but does not clarify system authorization responsibilities or security boundary expectations. Recommendation: We recommend clarifying whether deployed systems will operate under existing Government ATOs or require contractor-supported authorization packages and defining RMF documentation responsibilities. Value: Clarifying these requirements will align cybersecurity compliance activities with deployment schedules and reduce delays associated with security authorization processes.	19-Mar-26	The contractor will not be responsible for authorization packages or defining RMF documentation responsibilities. However, the contractor shall comply with any implemented security controls for the product being deployed.
87	11-Mar-26	52.212-1&-2	1	Solicitation Information	Lines 7–10	Observation: The solicitation indicates awards will be made to all technically acceptable offerors and that price will not be evaluated. Recommendation: We recommend clarifying the minimum criteria used to determine technical acceptability across each evaluation factor. Value: Providing additional clarity regarding evaluation thresholds will ensure consistent proposal evaluation and improve alignment between proposal responses and Government expectations.	19-Mar-26	52.212-2 provides the minimum criteria used to determine technical acceptability for each evaluation factor.

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88	11-Mar-26	52.212-1&-2	4	3.1 Page Limitations	Lines 157–166	Observation: The solicitation establishes strict page limitations for proposal volumes but does not clarify how graphical content is treated within those limits. Recommendation: We recommend clarifying whether graphics, diagrams, architectural visuals, and tables count toward page limits when used to illustrate technical approaches as well as if smaller font is for graphical content is acceptable . Value: Clarification will ensure consistent proposal formatting across offerors.	19-Mar-26	88a. 52.212-1 Section 3.1 provides for the use of tables, charts, graphs and figures in the proposal in response to the evaluation factors. These are considered to be part of the offeror's proposal and are included as part of the maximum page limits. 88b. 52.212-1 Section 3.2 states that The font size limitation applies to all narrative elements, graphics, art, figures, imported charts, charts, and any other graphical representation that includes text (font type can be varied depending on the medium).
89	11-Mar-26	52.212-1&-2	2	Proposal Submission Instructions	Lines 96–99	Observation: The solicitation indicates that artificial intelligence tools may be used during proposal evaluation to summarize proposal content. Recommendation: We recommend clarifying whether AI tools will be used solely for administrative summarization or will assist evaluators in identifying evaluation themes. Value: Greater transparency regarding the role of AI will ensure fairness and maintain confidence in the evaluation process.	19-Mar-26	See question 6.
90	11-Mar-26	52.212-1&-2	2	Proposal Submission Portal	Lines 65–90	Observation: The solicitation requires proposal submission through the Bidscale portal but does not specify contingency procedures in the event of system outages or submission failures. Recommendation: We recommend clarifying whether alternate submission procedures will be authorized under such circumstances. Value: Providing contingency procedures will ensure fair submission opportunities and reduce risk associated with technical submission issues.	19-Mar-26	Offerors are responsible for completing proposal submissions before the closing date and time; therefore, early submissions are encouraged. Contingency instructions are located in 52.212-1 Section 2.2 Paragraph 3.
91	11-Mar-26	52.212-1&-2	5	Factor 1 Challenge Scenario	Lines 219–238	Observation: The solicitation requires deployment approaches for both Federal EHR modernization and Operational Medicine deployments but does not specify the expected level of operational detail. Recommendation: We recommend clarifying whether responses should emphasize conceptual deployment methodologies or detailed execution plans including staffing structures, logistics planning, and implementation timelines. Value: Clarification will help ensure proposals are aligned with Government expectations for the challenge scenario evaluation.	19-Mar-26	91a. Offerors should propose their deployment methodology/approach based on CDRL A003 and the Sub-Factors listed in the 52.212-2. 91b. The detail of the offerors proposed approach should be based on the details provided in 52.212-1 and 52.212-2, Factor 1 Challenge Scenario, the offerors proposed assumptions, and page limitations (52.212-1, Section 3).
92	11-Mar-26	52.212-1&-2	6	Factor 2 Small Business Participation	Lines 254–283	Observation: The solicitation requires evidence of subcontractor participation commitments but does not clarify the form of documentation required. Recommendation: We recommend clarifying whether letters of commitment are required at the time of proposal submission or whether subcontractors may be proposed without formal commitment documentation. Value: Clarification will help offerors structure compliant teaming arrangements while supporting small business participation goals.	19-Mar-26	52.212-1 provides offerors to include Teaming Agreements as part of the Contract Document Volume. For Small Business Participation see question 36.
93	11-Mar-26	CDRL A003	1	Deployment Approach Description	General	Observation: The Exhibit requires submission of a deployment approach but does not clarify the level of methodological detail expected. Recommendation: We recommend clarifying whether responses should address phased implementation activities such as planning, site preparation, deployment execution, validation, and post-deployment operational support. Value: Providing this guidance will ensure offerors present comprehensive deployment strategies aligned with Government expectations.	19-Mar-26	In the context of proposing to 52.212-1 Factor 1 Challenge Scenario, offerors shall propose the necessary details to meet the minimum evaluation criteria listed in 52.212-2.

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94	11-Mar-26	CDRL A003	1	Deployment Planning	General	Observation: The Exhibit requires a deployment approach but does not specify whether estimated deployment timelines or milestone schedules should be included. Recommendation: We recommend clarifying expectations regarding deployment timelines, sequencing of activities across sites, and coordination with healthcare operational schedules. Value: Providing this clarification will allow offerors to present realistic implementation timelines that minimize operational disruption.	19-Mar-26	Specificity such as this may vary by product and task order. In the context of proposing to 52.212-1 Factor 1 Challenge Scenario, offerors shall propose estimated deployment timelines based on the scenario provided, the offeror's stated assumptions, and the offerors proposed response.
95	11-Mar-26	CDRL A003	1	Deployment Risk Management	General	Observation: The Exhibit requires a deployment approach but does not explicitly reference risk management planning. Recommendation: We recommend clarifying whether responses should include risk identification frameworks, mitigation strategies, and contingency planning. Value: Including risk management guidance will help ensure deployments are resilient to operational and technical disruptions.	19-Mar-26	Pursuant to 52.212-2 Factor 1 Challenge Scenario, Sub-Factor 1 Program Management and Governance, offerors shall address risk management.
96	11-Mar-26	CDRL A003	1	Technical Environment Integration	General	Observation: The Exhibit references deployment within existing medical environments but does not clarify expectations regarding infrastructure integration. Recommendation: We recommend clarifying expectations for network configuration, hardware integration, and interoperability with existing medical systems. Value: Providing this information will help ensure deployment approaches support safe integration into operational healthcare environments.	19-Mar-26	This is clarified in PWS Section 3.3.1 Network Integration and Configuration and not part of CDRL A003. Based on industry feedback, the Government is revising PWS Section 3. Specificity such as this may vary by product and task order.
97	11-Mar-26	CDRL A003	1	User Adoption Considerations	General	Observation: The Exhibit focuses on deployment execution but does not clarify whether user adoption and change management activities should be included. Recommendation: We recommend clarifying whether training, adoption support, and change management activities should be addressed within the deployment approach. Value: Incorporating adoption strategies into deployment planning will improve user acceptance and system utilization.	19-Mar-26	See box 16 of A003.
98	11-Mar-26	CDRL A003	1	Multi-Site Deployment Coordination	General	Observation: The Exhibit requires a deployment approach but does not specify expectations for coordinating deployments across multiple locations. Recommendation: We recommend clarifying whether responses should address concurrent deployments, centralized program management oversight, and coordination across geographically distributed medical facilities. Value: Providing this clarification will help ensure offerors propose scalable program management structures capable of supporting enterprise-wide deployments.	19-Mar-26	Specificity such as this may vary by product and task order. In the context of proposing to 52.212-1 Factor 1 Challenge Scenario, offerors shall review the scenarios provided and provide an approach based on the offeror's business decisions.
99	11-Mar-26	CDRL A003	N/A	N/A	N/A	Document was not available to download or render	19-Mar-26	Other offerors were able to download this document. This issue is often related to browser settings for opening PDFs. For the combined synopsis/solicitation, should this occur try downloading the PDF and then opening in the adobe application.

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100	11-Mar-26	PWS	2	Purpose and Scope	51-53	The PWS identifies a broad set of integrated deployment services including program management, configuration, training, adoption, and data migration. Does the Government anticipate issuing single task orders covering end-to-end deployment services, or separate task orders for individual functional areas (e.g., program management, deployment execution, training/adoption)? If separate task orders are contemplated, how will the Government ensure integration accountability across vendors, particularly where configuration, workflow design, and training must align to the underlying product selection?	19-Mar-26	The Government anticipates task order awardees to provide the end-to-end services required for a specific deployment, which will be delineated in the specific task orders. See question 3.